Activity 9: Employee Feedback and Listening System

Learning outcome: LO3 Objective: Gather safe, representative employee feedback using complementary methods Expected output: Multi-method listening plan, draft instruments and close-the-loop protocol Primary source: https://workplacementalhealth.org/mental-health-topics/resilience

Goal

Design a feedback system that combines surveys, focus groups, interviews and programme data while protecting identity and closing the loop.

Real-world scenario

The Center for Workplace Mental Health describes organisational resilience through leadership, training, a culture of empowerment, purpose, trust and accountability, supportive work environments, flexibility and access to support. Employee voice is needed to understand whether those conditions are experienced in practice.

RiverTech Digital, a fictional hybrid employer, wants quarterly feedback after a wellbeing launch. Employees distrust a survey that collects team, age, disability, free text and email. Managers want raw comments by team.

Evidence boundary

Public case facts and fictional learner data must never be mixed. Every numerical or organisational training input is labelled as an assumption.

Training assumptions / guardrails

- All sample responses and organisational details are fictional.
- Reporting groups must meet a minimum threshold and free text needs disclosure review.
- An independent research administrator is available for confidential interviews.
- Immediate safety disclosures require a defined, transparent escalation route.

Questions

1. Which question requires survey, focus group, interview or service data?
2. What data should not be collected, and what reporting threshold is safe?
3. How will employees understand anonymity, confidentiality and escalation?
4. How will management acknowledge, decide and close the feedback loop?

Detailed activity procedure

Step 1: Define listening questions

Write a small set of decisions the feedback must inform: access, trust, quality, work conditions or outcome. Remove curiosity-only collection.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 2: Choose complementary methods

Match survey, focus group, interview and programme data to each question. Record strengths, bias, burden and privacy risk.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 3: Design sampling

Identify who must be heard, recruitment route, participation barriers, minimum cell sizes and actions for underrepresented groups.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 4: Draft instruments

Use neutral, single-focus questions, appropriate scales and optional free text. Avoid clinical screening unless qualified governance exists.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 5: Write the privacy notice

Explain purpose, voluntariness, data fields, administrator, recipients, aggregation, retention, withdrawal limits and safety escalation.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 6: Create the analysis protocol

Predefine denominators, suppression, coding, contradictory evidence, uncertainty and free-text redaction. Prohibit manager access to raw comments.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 7: Build the response workflow

Set acknowledgement, triage, urgent escalation, decision owner, action tracking and employee communication timelines.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Step 8: Pilot and refine

Run cognitive testing and a small pilot. Check comprehension, completion, disclosure risk and whether results can support the intended decision.

Checkpoint: Record the evidence, calculation or decision produced in this step.

Deliverable

Multi-method listening plan, draft instruments and close-the-loop protocol

Acceptance criteria

The system collects only decision-relevant data, uses complementary methods, protects small groups and free text, accurately distinguishes anonymity/confidentiality, includes safety escalation and commits to visible response timelines.

Reflection and debrief questions

1. Which assumption changed your recommendation most, and why?
2. What evidence would you request before relying on the result?

3. Which stakeholder owns the next decision or mitigation?
4. Which review trigger or future exercise should follow?

Source note

Source accessed 16 August 2026: https://workplacementalhealth.org/mental-health-topics/resilience